



Irritable Bowel Syndrome (IBS)

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Reviewed/Revised Apr 2024 | Modified Nov 2025

Irritable bowel syndrome is a disorder of the digestive tract that causes recurring abdominal pain and constipation or diarrhea.

- Symptoms vary but often include lower abdominal pain, bloating, gas, and constipation or diarrhea.
- A variety of substances and emotional factors can trigger symptoms of irritable bowel syndrome.
- A doctor usually diagnoses irritable bowel syndrome based on the symptoms but does tests to rule out other problems.
- Diet modification and medications can usually relieve specific symptoms.

Irritable bowel syndrome (IBS) is common among the general population. IBS is a common reason why people visit their primary care physician.

IBS is a disorder of the movement of the intestines, the sensitivity of the nerves of the intestines, or the way in which the brain controls some of these functions. However, although the normal functioning is impaired, there are no structural abnormalities that can be found with an endoscope (a flexible viewing tube), imaging studies, biopsies, or blood tests. Thus, IBS is identified by the characteristics of the symptoms and, when done, normal results of tests.

Causes of Irritable Bowel Syndrome

The cause of IBS is not clear. In many people with IBS, the digestive tract is especially sensitive to stimuli. People may experience discomfort caused by intestinal gas or contractions that other people do not find distressing. Although the changes in bowel movements that occur with IBS might seem to be related to abnormal intestinal contractions, not all people with IBS have abnormal contractions, and in many of those who do, the abnormal contractions do not always coincide with symptoms. In some people, symptoms of IBS begin after an episode of [gastroenteritis](#).

For some people, high-calorie meals or a high-fat diet may be a trigger.

For other people, wheat, dairy products, beans, chocolate, coffee, tea, some artificial sweeteners, certain vegetables (such as asparagus or broccoli), or stone fruits (such as apricots) seem to aggravate the symptoms. These foods contain carbohydrates that are poorly absorbed by the small intestine. The carbohydrates become fermented by bacteria in the intestine, which causes gas, bloating, and cramping. Because many food products contain several ingredients, it may be difficult to identify the specific trigger.

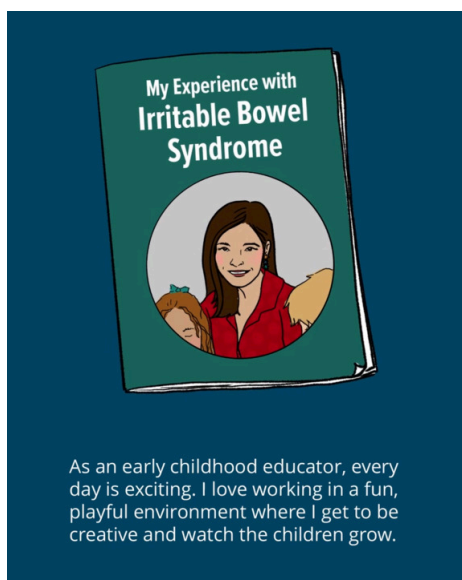
Other people find that eating too quickly or eating after too long a period without food stimulates a flare-up (a bout or attack). However, the relationship is often inconsistent.

Emotional factors (for example, stress, anxiety, depression, and fear), viral illnesses, or hormones may trigger or worsen a flare-up of IBS.

People do not always get symptoms after a usual trigger, and symptoms often appear without any obvious trigger. It is not clear how all the triggers relate to the cause of IBS.

Hidden Disability: Irritable B...

VIDEO



Symptoms of Irritable Bowel Syndrome

IBS tends to begin in adolescence and the 20s, causing bouts of symptoms that come and go at irregular periods. The start of IBS symptoms in late adult life is possible but less common. Flare-ups almost always occur when a person is awake, and they rarely wake a person from sleep.

Symptoms of IBS include [abdominal pain](#) related to or relieved by having a bowel movement (defecation). The abdominal pain is associated with a change in stool frequency (such as [constipation](#) or [diarrhea](#)) or consistency (loose or lumpy and hard). The pain may come in bouts of continuous dull aching or cramps, usually over the lower abdomen. Symptoms of IBS can also include abdominal expansion (distention), mucus in the stool, and the sensation of incomplete emptying after defecation.

Bloating, [gas](#), [nausea](#), headaches, fatigue, depression, anxiety, muscle aches, problems with sleep, and difficulty concentrating are other possible symptoms.

Diagnosis of Irritable Bowel Syndrome

- A doctor's evaluation based on the person's symptoms
- Some laboratory tests to look for other disorders

Most people with IBS appear healthy. Doctors base the diagnosis of IBS on the characteristics of the person's symptoms. Doctors also use standardized symptom-based criteria for diagnosing IBS called the Rome criteria. They may also do tests to diagnose common illnesses that can cause similar symptoms, particularly when people are over 45 or have warning signs such as weight loss, rectal bleeding, or older age.

Doctors use the Rome criteria to diagnose IBS in people who have had abdominal pain for at least 1 day a week in the last 3 months along with 2 or more of the following:

- Pain is related to defecation.
- Pain is associated with a change in stool frequency (constipation or diarrhea).
- Pain is associated with a change in the consistency of stool.

A physical examination generally does not reveal anything unusual except sometimes tenderness over the large intestine. Doctors do a digital rectal examination, in which a gloved finger is inserted in the person's rectum. Women also may undergo a [pelvic examination](#).

Doctors usually do some tests, for example, blood tests, to differentiate IBS from [Crohn disease](#), [ulcerative colitis](#), cancer (mainly in people over age 45), microscopic colitis, [celiac disease](#), and other diseases and infections that can cause abdominal pain and changes in bowel habits. These test results are usually normal in people with IBS.

Doctors may do other tests, such as [ultrasonography](#) of the abdomen or other imaging studies of the intestines, in people who have symptoms that are unusual for IBS, such as fever, bloody stools, weight loss, and vomiting. [Colonoscopy](#) usually is done in people over 45 years of age to rule out tumors or polyps in the large intestine.

Other digestive tract disorders (such as [appendicitis](#), [gallbladder disease](#), ulcers, and cancer) may develop in a person with IBS, particularly after age 45. Thus, if a person's symptoms change significantly, if new symptoms develop, or if symptoms are unusual for IBS, further testing may be needed.

Because IBS symptoms can be triggered by stress and emotional conflicts, doctors ask questions to help identify stress, anxiety, or mood disorders. Doctors also ask questions to rule out [laxative abuse](#).

Treatment of Irritable Bowel Syndrome

- Eating a normal diet and avoiding gas-producing and diarrhea-producing foods

- Increasing fiber and water intake for constipation
- Sometimes medications

Treatment of IBS differs from person to person. If particular foods or types of stress appear to trigger symptoms, they should be avoided if possible.

For most people, especially those who tend to be constipated, regular physical activity helps keep the digestive tract functioning normally.

Diet

(For more information about diet and IBS, see these [recommendations](#) from the National Institute of Diabetes and Digestive Disease.)

Many people do better eating frequent, smaller meals rather than less frequent, larger meals (for example, 5 or 6 small meals rather than 3 large meals a day). People should try to slow their pace while eating. People with bloating and increased gas (flatulence) should avoid beans, cabbage, and other foods that are difficult to digest.

Some people find relief from IBS symptoms by restricting their intake of foods that are high in certain [carbohydrates](#) called fermentable oligosaccharides, disaccharides, monosaccharides, and polyols. These foods are collectively called FODMAPs. FODMAPs are carbohydrates that are poorly absorbed and rapidly fermented by bacteria in the small intestine, leading to increased gas and discomfort.

Sorbitol, an artificial sweetener used in some foods and chewing gums, should not be consumed in large amounts. Fructose, a sugar found in fruits, berries, and some plants, should be eaten only in small amounts. People who have IBS and who cannot digest the sugar lactose (called [lactose intolerance](#)), which is found in milk and other dairy products, should consume dairy products only in moderation.

People can try reducing their intake of the foods mentioned above one at a time and noting whether their symptoms change, or they can try a low-FODMAP diet, which restricts all of these foods all at once. If changes in diet cause symptoms to lessen, people can gradually reintroduce restricted foods one at a time and monitor their symptoms.

A low-fat diet helps some people, particularly those whose stomach empties too slowly or too quickly.

Constipation can often be relieved by eating more fiber and drinking more water. People with constipation can take [psyllium](#) mucilloid supplements with 2 glasses of water. Increasing the dietary fiber may aggravate flatulence and bloating. Occasionally, such flatulence may be reduced by switching to a synthetic fiber preparation (such as [methylcellulose](#) or [psyllium](#)).

Medications

Certain laxatives are reasonably safe and often effective for people with constipation. Such laxatives include those containing [polyethylene glycol](#) and stimulant laxatives, such as those containing [bisacodyl](#) or [glycerin](#). The prescription laxatives [lubiprostone](#), [linaclotide](#), [plecanatide](#), and [tenapanor](#) may also

relieve constipation. [Prucalopride](#) is another medication that affects motility of the intestines and may help people who have chronic constipation.

Anticholinergic medications, such as [hyoscyamine](#), can sometimes relieve abdominal pain by blocking spasms of the intestinal muscles. However, these medications often cause anticholinergic side effects (see sidebar [Anticholinergic: What Does It Mean?](#)), such as dry mouth, blurred vision, or difficulty urinating.

Antidiarrheal medications, such as diphenoxylate or [loperamide](#), help people with diarrhea. [Eluxadoline](#) is another medication that may be given to some people who have severe diarrhea caused by IBS.

The **antibiotic** [rifaximin](#) may be prescribed to relieve symptoms of diarrhea, bloating, and abdominal pain.

[Alosetron](#) is occasionally used for diarrhea in older women for whom other medications are ineffective. However, alosetron has been associated with increasing the risk of ischemic colitis, so its use is restricted in the United States.

Certain antidepressants help relieve symptoms of abdominal pain as well as diarrhea and bloating in many people. Long-term use of certain antidepressants such as [nortriptyline](#) or [desipramine](#) is often helpful. Antidepressants may not only relieve pain and other symptoms but also may help relieve sleep problems and depression or anxiety.

Probiotics, which are bacteria naturally found in the body that promote the growth of good bacteria, may be given.

Aromatic oils, such as peppermint oil, often help relieve pain caused by cramps in some people.

Other treatments

Behavior modification techniques (such as cognitive-behavioral therapy), [psychotherapy](#), and [hypnotherapy](#) (hypnosis) may help some people who have IBS.

More Information

The following English-language resources may be useful. Please note that THE MANUAL is not responsible for the content of these resources.

[International Foundation for Functional Gastrointestinal Disorders \(IFFGD\)](#): Education, assistance, and support for people affected by gastrointestinal (GI) disorders

[National Institutes of Health \(NIH\)](#): Eating, diet (including FODMAP diet), and nutrition information for irritable bowel syndrome

Drug Information for the Topic
